



Patient NAME : Mr.MANOJ KUKREJA
Age/Gender : 57 Y 0 M 0 D /Male
Visit ID : HVD165665
Barcode NO : U2242873
Ref Doctor :SELF
CLIENT CODE :DEL-53



CLIENT NAME : DEL-53
Sample Collection : 28/Mar/2026 10:57AM
Registration : 28/Mar/2026 10:45AM
Reported : 28/Mar/2026 12:26PM



DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
Complete blood count with Esr (CBC)				
Hemoglobin(HB)	12.8	g/dl	13.0-18.0	Photometric/Non Cyanmethemoglobin Method
Total Leucocytes Count(WBC)	5,540	Cells/Cumm	4000-10500	Automated optical Flow cytometry/Manual
Total Count and Differential Count				
Neutrophils	59	%	40-80	Impedance/microscopy
Lymphocytes	35	%	20-40	Impedance/microscopy
Eosinophils	4	%	01-06	Impedance/microscopy
Monocytes	2	%	02-10	Impedance/microscopy
Basophils	0	%	00-01	Impedance/microscopy
Absolute Neutrophil Count	3,269	x10 ³ Cells/uL	2000-8000	Automated Calculated
ABSOLUTE LYMPHOCYTE COUNT	1,939	x10 ³ Cells/uL	1000-3000	Automated Calculated
Absolute Eosinophil Count(AEC)	222	Cells/cumm	20-500	Automated Calculated
ABSOLUTE MONOCYTE COUNT	111	x10 ³ Cells/uL	200-1000	Automated Calculated
Mean Cell Haemoglobin (MCH)	30.5	pg	27-32	Automated Calculated
MCHC	34.1	g/dl	31.5-34.5	Automated/Calculated
Erythrocyte count (RBC COUNT)	4.20	million/cmm	3.8-4.8	Impedance
Packed Cell Volume(Hematocrit)	37.40	%	36-46	Cell Counter
Mean Cell Volume (MCV)	89.30	fL	83-101	Automated/Calculated
Red Cell Distribution Width (RDW)-SD	52.70	fL	35-56	Automated/Calculated

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DEPARTMENT OF HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
Red Cell Distribution Width (RDW)-CV	15.40	%	11.5-14.5	Automated/Calculated
Platelet Count	2.40	Lakh/cumm	1.50 - 4.50	Impedance/microscopy
Plateletcrit (PCT)	0.20	%	0.2-0.5	Automated Optical Flowcytometer
Platelet-Large Cell Count (P-LCC)	54.00	lakh/cmm	40-100	Automated Calculated
Platelet large cell ratio (P-LCR)	53.80	%	11.9-66.9	Automated/Calculated
Platelet Distribution Width (PDW-CV)	12.80	%	9.00-17.00	Automated/Calculated
Platelet Distribution Width (PDW-SD)	12.90		0-25	Automated/Calculated
Platelet-to-Lymphocyte Ratio(PLR)	124.00		36.63-149.13	Automated/Calculated
Erythrocytes Sedimentation Rate (ESR)	8.00	mm/1st hr	0-10	Westergren

Interpretation - There have been some reports of WBC and platelet counts being lower in venous blood than in capillary blood samples,although still within these reference ranges.

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CLIENT NAME : DEL-53
Sample Collection : 28/Mar/2026 10:57AM
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Reported : 28/Mar/2026 11:54AM



DEPARTMENT OF BIOCHEMISTRY

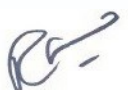
Test Name	Result	Unit	Bio. Ref. Range	Method
GLUCOSE- FASTING (FBS)				
Glucose- Fasting	151.90	mg/dl	Non Diabetic: 70-106 mg/dL Prediabetes: 106-125 mg/dL Diabetes Mellitus: 126 or higher	GOD-POD,END POINT

INTERPRETATION:
Increased In

- Diabetes Mellitus
- Stress (e.g., emotion, burns, shock, anesthesia)
- Acute pancreatitis
- Chronic pancreatitis
- Wernicke encephalopathy (vitamin B1 deficiency)
- Effect of drugs (e.g. corticosteroids, estrogens, alcohol, phenytoin, thiazides)

CLINICAL COMMENT :

Elevated glucose levels (Hyperglycemia) are mostly associated with Diabetes mellitus but may be observed in case of neoplasms, hyperthyroidism, pancreatic and adrenocortical disorders. Hyperglycemia can also be due to effect of drugs (e.g. corticosteroids, estrogens, alcohol, phenytoin, thiazides). Decreased glucose levels (Hypoglycemia) may occur due increased insulin secretion, prolonged fasting or liver disease.


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Sample Collection : 28/Mar/2026 10:57AM

Registration : 28/Mar/2026 10:45AM

Reported : 28/Mar/2026 12:03PM



DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
LIVER FUNCTION TEST				
Total Bilirubin	0.62	mg/dL	0.0-1.0	DCA
Conjugated (D. Bilirubin)	0.14	mg/dL	0.0-0.2	DCA
Unconjugated (I.D.Bilirubin)	0.48	mg/dl	0.0-0.8	Calculated
Alkaline Phosphatase	91.60	U/L	53-128	IFCC,KINETIC
Alanine Aminotransferase(ALT/SGPT)	22.80	U/L	<40	IFCC
Aspartate Transaminase (AST/SGOT)	24.10	U/L	0.0-37.0	IFCC
Gamma Glutamyl Transferase(GGT)	32.50	U/L	Upto 60	g-Glut-3-carboxy-4-nitro
Total Protein	6.70	gm/dl	6.0-8.0	Biuret
Serum Albumin	4.20	g/dl	3.7-5.3	Bromocresol Green (BCG)
Seum Globulin	2.50	g/dl	2.3-3.6	Calculated
Albumin/Globulin Ratio	1.68	Ratio	1.0-2.1	Calculated

Comments - Liver Function Tests (LFTs) are group of tests that can be used to detect the presence of liver disease, distinguish among different types of liver disorders, gauge the extent of known liver damage, and monitor the response to treatment. Most liver diseases cause only mild symptoms initially, but these diseases must be detected early. Some tests are associated with functionality (e.g., albumin), some with cellular integrity (e.g., transaminase), and some with conditions linked to the biliary tract (gamma-glutamyl transferase and alkaline phosphatase). Conditions with elevated levels of ALT and AST include hepatitis A,B ,C ,paracetamol toxicity etc.Several biochemical tests are useful in the evaluation and management of patients with hepatic dysfunction. Note : The result obtained relate only to the sample given/ received & tested. A single test result is not always indicative of a disease, it has to be correlated with clinical data for interpretation.

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
KFT+ELECTROLYTES				
Blood Urea	32.80	mg/dL	14-40	GLDH KINETIC METHOD
Blood Urea Nitrogen(BUN)	15.33	mg/dl	5-25	Calculated
Serum Creatinine	0.91	mg/dl	0.6-1.2	Enzymatic
Bun/Creatinine Ratio	16.85		6-23	Calculated
Serum Uric Acid	5.20	mg/dL	3.4-7.0	PAP
Calcium	8.70	mg/dl	8.6-10.2	ARSENazo III
Sodium	136.20	mmol/L	135-155	ISE Direct
Potassium	4.10	mmol/L	3.5-5.0	ISE Direct
Chloride	102.30	mmol/L	95-108	ISE Direct
GFR by MDRD Formula	91.4		90 - 120 mL/min/1.73 m2	Calculated

COMMENTS- Urea is a non-proteinous nitrogen compound formed in the liver from ammonia as an end product of protein metabolism. Increased levels are found in acute renal failure, chronic glomerulonephritis, congestive heart failure, decreased renal perfusion, diabetes, excessive protein ingestion, gastrointestinal (GI) bleeding, hyperalimentation, hypovolemia, ketoacidosis, muscle wasting from starvation, neoplasms, pyelonephritis, shock, urinary tract obstruction, nephrotoxic drugs. Decreased levels are seen in inadequate dietary protein, low-protein/high-carbohydrate diet, malabsorption syndromes, pregnancy, severe liver disease and certain drugs. Creatinine is catabolic product of creatinine phosphate, which is excreted by filtration through the glomerulus and by tubular secretion. Creatinine clearance is an acceptable clinical measure of glomerular filtration rate (GFR). Increased levels are seen in acute/chronic renal failure, urinary tract obstruction, hypothyroidism, nephrotoxic drugs, shock, dehydration, congestive heart failure, diabetes. Decreased levels are found in muscular dystrophy. BUN is directly related to protein intake and nitrogen metabolism and inversely related to the rate of excretion of urea. Blood urea nitrogen (BUN) levels reflect the balance between the production and excretion of urea. Increased levels are seen in renal failure (acute or chronic), urinary tract obstruction, dehydration, shock, burns, CHF, GI bleeding, nephrotoxic drugs. Decreased levels are seen in hepatic failure, nephrotic syndrome, cachexia (low-protein and high-carbohydrate diets). BUN/Creatinine ratio is decreased in acute tubular necrosis, advanced liver disease, low protein intake and following hemodialysis. BUN/Creatinine ratio is increased in dehydration, GI bleeding, and increased catabolism. Uric acid levels show diurnal variation. The level is usually higher in the morning and lower in the evening. Increased levels are seen in starvation, strenuous exercise, malnutrition, or lead poisoning, gout, renal disorders, increased breakdown of body cells in some cancers (including leukemia, lymphoma, and multiple myeloma) or cancer treatments, hemolytic anemia, sickle cell anemia, or heart failure, pre-eclampsia, liver disease (cirrhosis), obesity, psoriasis, hypothyroidism, low blood levels of parathyroid hormone (PTH), certain drugs, foods that are very high in purines - such as organ meats, red meats, some seafood and beer. Decreased levels are seen in liver disease, Wilson's disease, Syndrome of inappropriate antidiuretic hormone (SIADH), certain drugs. Electrolyte profile(* profile is not a scope of nabl) disturbance showing Extreme fatigue. A prolonged bout of diarrhea or vomiting. Signs of dehydration. Unexplained confusion, muscle cramps, numbness or tingling. certain electrolyte is too high, the kidney might try to release more of it in your urine. Electrolyte imbalances can cause problems with many

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
LIPID PROFILE				
Total Cholesterol	180.80	mg/dl	Desirable:<200 Borderline:200-239 High risk:>240	Spectrophotometry
Triglycerides	116.10	mg/dl	Normal:<150 BorderLine:150-199 High:200-499 Very High:>500	PAP
Cholesterol-HDL	55.20	mg/dl	Low:<35 Optimal:35-60 Desirable:>80	Direct measure-PEG
Cholesterol-L D L (Direct)	102.38	mg/dl	Normal:<100~Borderline High:130-159~Very High:>190	Calculated
Cholesterol- V L D L	23.22	mg/dl	7-40	Calculated
Total Cholesterol /HDL Ratio	3.28		<6	Calculated
LDL / HDL Ratio	1.85	Ratio	0.0-3.5	Calculated

COMMENTS: Measurements in the same patient can show physiological & analytical variations. Three serial samples 1week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol. As per NCEP guidelines, all adults above the age of 20 years should be screened for lipid status. NCEP identifies elevated Triglycerides as an independent risk factor for Coronary Heart Disease (CHD). ATP III guidelines uses LDL Cholesterol as the primary target for Cholesterol lowering therapy. Note that major risk factors can modify LDL goals

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
IRON DEFECIENCY PROFILE				
Iron	94	µg/dL	59-158	FerroZine-without deproteinization.
Iron Binding Capacity - Total (TIBC)	329	µg/dL	250-450	Spectrophotometry
UIBC	234.60	ugm/dL	130 - 336	Ferrozine
Transferrin Saturation	28.6	%	20-50	Calculation

General Age-Based Ranges

- **Newborns:** 100–250 µg/dL
- **Children (general):** 50–120 µg/dL
- **Children (2-12 years):** 22–135 µg/dL
- **Children (6 months):** 28–135 µg/dL

INTERPRETATION:

SERUM IRON INCREASED IN:

Hemosiderosis of excessive iron intake (e.g. repeated blood transfusion, iron therapy, iron containing vitamins).Decreased formation of RBCs (thalassemia, pyridoxal deficiency anaemia).Increased destruction of RBCs (hemolytic anaemia).Acute liver damage.Acute iron toxicity.

SERUM IRON DECREASED IN:

Iron deficiency anaemia, Normochromic anaemia of infections & chronic diseases, Nephrosis, Menorrhagia, Diurnal variation: Normal in mid morning, low values in mid afternoon, and very low values near midnight.

TIBC/UIBC INCREASED IN:

Iron deficiency anemia, Acute & Chronic blood loss, Acute liver damage, Progesterone birth control pills.

TIBC/UIBC DECREASED IN:

Hemochromatosis, Cirrhosis of the liver, Thalassemia, Anemia of infective & chronic disease, Nephrosis.

TRANSFERRIN SATURATION INCREASED IN:

High Values in iron overload, Raised transferrin saturation is an early indicator of Iron accumulation in hemochromatosis.

TRANSFERRIN SATURATION DECREASED IN:

Low Values in iron deficiency.

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DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Range	Method
Glycosylatd Hemoglobin (GHB/HbA1C)				
Glycosylated Hemoglobin (GHB/HbA1C)	11.90	%	Non Diabetic:<6 Excellent control:6-7 Fair to Good control:7-8 Unsatisfactory control:8-10 Poor Control:>10	High-performance Liquid Chromatography (HPLC)
Estimated Average Glucose	294.83	mg/dl	Excellent Control : 90-120 Good Control:121-150 Average Control:151-180 Action Suggested:181-210 Poor Control:>211	Calculated

1. HbA1C has been endorsed by clinical groups and American Diabetes Association guidelines 2017 for diagnosing diabetes using a cut off point of 6.5%
2. Low glycated haemoglobin in a non diabetic individual are often associated with systemic inflammatory diseases, chronic anaemia (especially severe iron deficiency and haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.
Excellent control-6-7 %
Fair to Good control – 7-8 %
Unsatisfactory control – 8 to 10 %
Poor Control – More than 10 %

INCREASED IN

1. Chronic renal failure with or without hemodialysis.
2. Iron deficiency anemia. Increased serum triglycerides

DECREASED IN

1. Shortened RBC life span (hemolytic anemia, blood loss), Pregnancy.
2. Ingestion of large amounts (>1g/day) of vitamin C or E.

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NABL RECOGNISED LAB VIDE CERTIFICATE NO. MT-02600

LABORATORY REPORT

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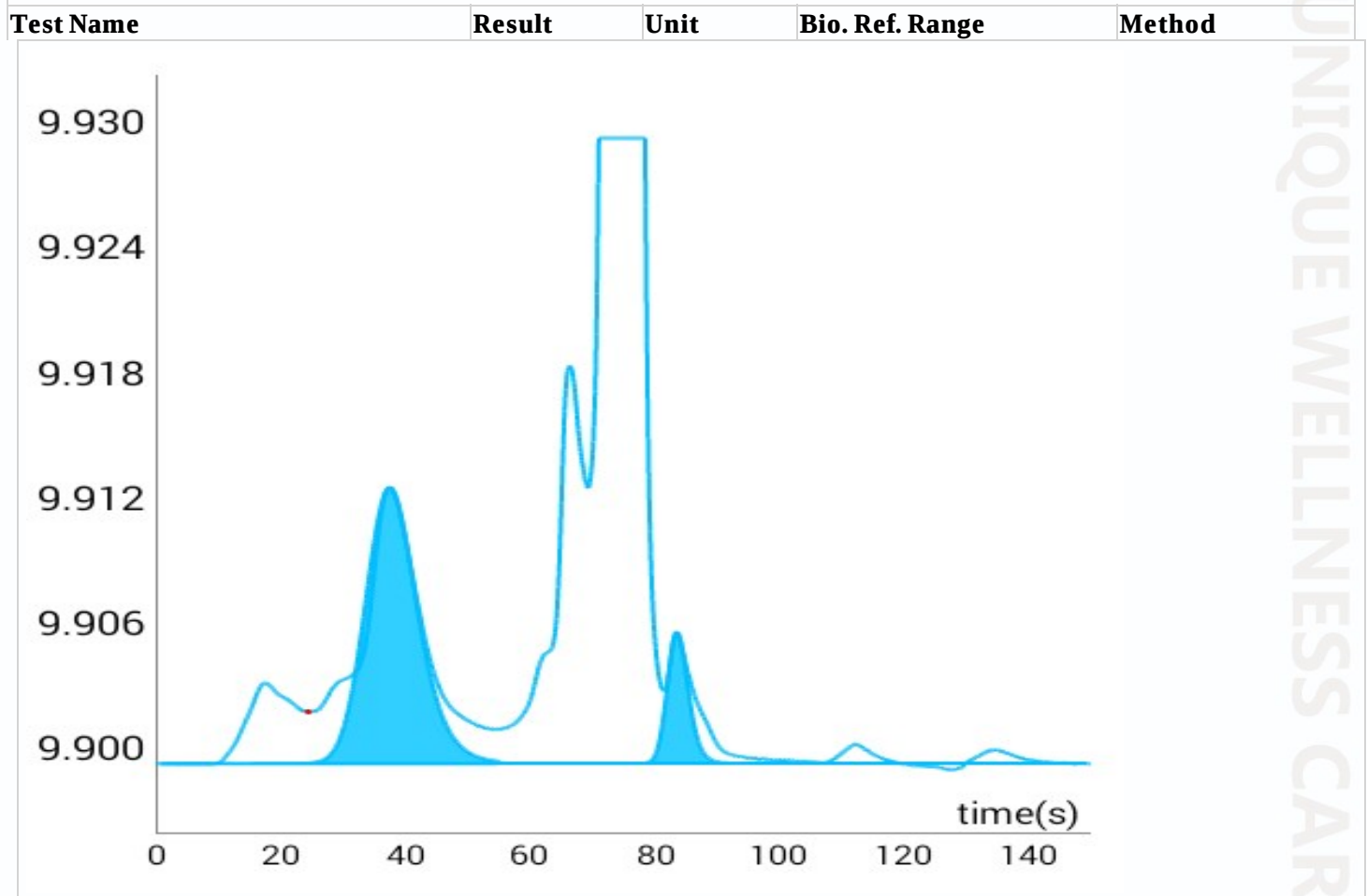
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DEPARTMENT OF BIOCHEMISTRY



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DEPARTMENT OF HORMONE ASSAYS

Test Name	Result	Unit	Bio. Ref. Range	Method
25-Hydroxy Vitamin D3 Total				
25-Hydroxy Vitamin D Total	27.10	ng/ml	Deficient : <20 Insufficient : 20 to <30 Sufficient : 30-100 Upper Safety Limit : >120	Chemiluminescence

INTERPRETATION:

LEVEL	REFERENCE RANGE
Deficiency (serious deficient)	< 20 ng/ml
Insufficiency (Deficient)	20-30 ng/ml
Sufficient (adequate)	30-100 ng/ml
Upper Safety Limit	>100 ng/ml

DECREASED LEVELS:

Deficiency in children causes Rickets and in adults leads to Osteomalacia. It can also lead to Hypocalcemia and Tetany. ,Inadequate exposure to sunlight.,Dietary deficiency.,Vitamin D malabsorption.,Severe Hepatocellular disease.,Drugs like Anticonvulsants.,Nephrotic syndrome.

INCREASED LEVELS:

-Vitamin D intoxication.

COMMENTS:

-Vitamin D (Cholecalciferol) promotes absorption of calcium and phosphorus and mineralization of bones and teeth. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1, 25 Dihydroxy vitamin D (5-8 hrs)..0

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DEPARTMENT OF HORMONE ASSAYS

Test Name	Result	Unit	Bio. Ref. Range	Method
VITAMIN B12				
Vitamin B12	299	pg/mL	Deficiency:< 145 Indeterminate:145 -180 Normal: 180 - 914	CLIA

Vitamin B12, also known as cyanocobalamin, is a water soluble vitamin that is required for the maturation of erythrocytes and coenzyme form for more than 12 different enzyme systems. Groups at risk for vitamin B12 deficiency include those

(1) older than 65 years of age (2) with malabsorption (3) who are vegetarians (4) with autoimmune disorders (5) taking prescribed medication known to interfere with vitamin absorption or metabolism, including nitrous oxide, phenytoin, dihydrofolate reductase inhibitors, metformin, and proton pump inhibitors (6) infants with suspected metabolic disorders.

COMMENTS:

Results may differ between laboratories due to variation in population and test method. Vitamin B12 is implicated in the formation of myelin, and along with Folate is required for DNA synthesis. The most prominent source of B12 for humans is meat while untreated fresh water can also be a source.

Megaloblastic anaemia has been found to be due to B12 deficiency, a major cause being Pernicious anemia due to poor B12 uptake resulting in below normal serum levels. Other conditions related to low B12 levels include iron deficiency anemia, pregnancy, vegetarianism, partial gastrectomy, ileal damage, oral contraceptives, parasitic infestations, pancreatic deficiency, treated epilepsy and advancing age. The correlation of serum B12 levels and Megaloblastic anemia however is not always clear - some patients with high MCV may have normal B12 levels, while some individuals with B12 deficiency may not have megaloblastic anemia. Disorders renal failure, liver diseases and myeloproliferative diseases may have elevated vitamin B12 levels.

LIMITATIONS:

For diagnostic purposes, the B12 results should be used in conjunction with other data; e.g.; symptoms results of other testing, clinical impressions, etc. If the B12 level is inconsistent with clinical evidence, additional testing is suggested to confirm the result.

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DEPARTMENT OF HORMONE ASSAYS

Test Name	Result	Unit	Bio. Ref. Range	Method
THYROID PROFILE I (T3, T4, TSH)				
Tri-Iodothyronine Total (T3)	156	ng/dL	>18 yrs: 60 -181 Pregnancy 1st Trimester: 81-91 2nd &3rd trimester:100-260	Chemiluminescence
Thyroxine Total (T4)	13.50	µg/dL	4.82-15.65	Chemiluminescence
TSH - ULTRASENSITIVE	1.6	µIU/mL	0.35-5.50	Chemiluminescence

INTERPRETATION:

Serum T3, T4 and TSH are the measurements form three components of thyroid screening panel and are useful in diagnosing various disorders of thyroid gland function., Primary hyperthyroidism is accompanied by elevated serum T3 and T4 values along with depressed TSH levels., Primary hypothyroidism is accompanied by depressed serum T3 and T4 values and elevated serum TSH levels., Normal T4 levels accompanied by high T3 levels are seen in patients with T3 thyrotoxicosis. Slightly elevated T3 levels may be found in pregnancy and in estrogen therapy while depressed levels may be encountered in severe illness, malnutrition, (**References range recommended by the American Thyroid Association**)

REFERENCE RANGE:

PREGNANCY	Ultrasensitive TSH in uIU/mL
1st Trimester	0.100 – 2.500
2nd Trimester	0.200 – 3.000
3rd Trimester	0.300 – 3.000

Age	Ultrasensitive TSH in uIU/mL
0 – 4 Days	1.00 - 39.00
2 Weeks to 5 Months	1.70 – 9.10
6 Months to 20 Yrs.	0.70 – 6.40
>55 Yrs.	0.50 - 8.90

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CLIENT NAME : DEL-53
Sample Collection : 28/Mar/2026 10:58AM
Registration : 28/Mar/2026 10:45AM
Reported : 28/Mar/2026 01:29PM



DEPARTMENT OF CLINICAL PATHOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
Complete Urine Examination (CUE))				
SAMPLE TYPE:URINE				
PHYSICAL EXAMINATION				
Colour	P.YELLOW		PALE YELLOW	
Volume	25.0	ml/ day		
Appearance	Clear			
CHEMICAL EXAMINATION				
pH	6.0		New Born:5.0-7.0 5.0-8.5	
Specific gravity	1.010		<1.020:Transudates >1.020:Exudates	
Proteins	NIL			
Glucose	Trace			
MICROSCOPIC EXAMINATION				
RBCs	NIL		NIL	Microscopy
Pus cells	1-2	Cells/Hpf	NIL	
Epithelial Cells	1-2	Cells/Hpf	NIL	Microscopy
Urine Crystals				
Calcium Oxalate	NIL		NIL	
Triple Phosphate	NIL		NIL	
Uric acid	NIL		NIL	
Calcium Carbonate Crystals	NIL		NIL	
Amorphous				
Amorphous Urates	NIL		NIL	
Amorphous Phosphates	NIL		NIL	

DR.RAM SHRAN
MBBS, MD (PATHOLOGY)
DMC -44740

DR ABDUL SAMEER QURESHI
MBBD,D.C.P
DMC-39510

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Note: Assay results should be correlated clinically with other clinical findings and the total clinical status of the patient.

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Patient NAME : Mr.MANOJ KUKREJA

Age/Gender : 57 Y 0 M 0 D /Male

Visit ID : HVD165665

Barcode NO : U2246135

Ref Doctor :SELF

CLIENT CODE :DEL-53



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DEPARTMENT OF CLINICAL PATHOLOGY

Test Name	Result	Unit	Bio. Ref. Range	Method
Casts				
Granular Cast	NIL		NIL	
Hyline cast	NIL		NIL	
RBC Cast	NIL		NIL	
WBC Cast	NIL		NIL	
Epithelial Cast	NIL		NIL	
Mucus Threads	NIL		NIL	
Bacteria	NIL		NIL	
Yeast Cells	NIL		NIL	
Spermatozoa	NIL		NIL	

*** End Of Report ***

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